

العمر(تاريخ الميلاد) : 04-05-1972 المهنة : بدون عمل

التوقيع: (_____)

التعليق على التشخيص : -----

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- / د ----- / د ----- / د. | ----- / د. |

التشخيص النهائي :
 خطة العلاج :
 المستشفى :
 قسم :
 نفقات :
 ملاحظات :

**رئيس
المجلس الطبي المتخصص**

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Name: هناء ابراهيم محمد عامر National ID: 27205041301948	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

هنا
ابراهيم محمد عامر
حوض نجيع
مركز ههيا . الشرقية

١٩٧٢/٠٥/٠٤ ٢٧٢.٥.٤١٣.١٩٤٨
HV4407056

٢٠١٩/٠١ ٢٧٢.٥.٤١٣.١٩٤٨
'حاصلة علم ليسانس اداب'
انثى ثمة انمة
البطاقة سارية حتى ٢٠٢٦/٠١/١٢ ٨

صرونا
موجه
زوميتا
متوافق عليهم

علاج مرمون
علاج نعيم
زوميتا
تاكيرب
فوا انقاس



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : **د. نورا إبراهيم محمد عمار**

Sex:

3- Medical number:

Female

83024148

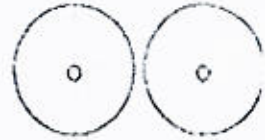
Old Med:

DC Med:

2798-2021

2-ID number: 27205041301948

Patient Age: 52 Y



Family history	Family history	Phone	PHONE	BMI	BMI	REASON OF MDT	REASON OF MDT
			01146224455				
marital status	Marital Status	NRS Score	NRS Score	GROUP	GROUP	Date of MDT	Date of MDT
							03/03/2
Co morbidity	Co Morbidity	Menopausal status	Menopausal Status	Initial STAGE	Initial STAGE		
			POST-MENOPAUSAL				

Breast Imaging :			Mets Workup :		
BX IOC BX COMMENT:	ER 8/8 ER COMMENT:	PR 7/8 PR COMMENT:	Her 2 Neu 3 Her 2 Neu COMMENT:	SISH SISH COMMENT:	Ki67
T			M 1bone L.N,pl,brain		
Sonomammography					
PATIENT HISTORY MBC to LN , BONE ,pleura & recently brain LT Bcs 2012-->6ac-->port-->TAM X 2 YRS 4/2020 PLEURAL EFFUSION -->ZOLADEX +ai-->ZOLA +fasoldex-->3 cycle taxol+herceptin-->RD--->afinitor 4 cycle-->RD-->aromasine+herceptin fro 3/2022 till 2/2023 brain mets ct:SD xeloda +tykerb with RD received 10 cycles xeloda+10 cycles tykerb ct 25/9/2023:SD mri BRAIN 10/2023:regression pt i receiving c11 xeloda+tykerb with RD of xeloda 2 x2 MRI brain:RD ct:free except SD of osseous lesion pt received 15 cycles xeloda+tykerb pt has grade 2-3 hand foot syndrom					
POST OUTCOME & PLAN Start Tamoxifen , Tykerb and Zometa every 3 ms. . NB ; No Suitable site for Re-biopsy.					

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د. نورا إبراهيم محمد عمار د. نورا إبراهيم محمد عمار	RADIOLOGY	RADIOLOGY د. نورا إبراهيم محمد عمار د. نورا إبراهيم محمد عمار
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

Name: هناة ابراهيم محمد عامر		
MR. : 3226		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالتها		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انها		
The patient is a 54-year-old post-menopausal individual with a history of metastatic breast cancer, HER2-positive, involving lymph nodes, pleura, bone and brain. The clinical course includes left breast-conserving surgery in 2011 followed by adjuvant AC ×6, postoperative radiotherapy and approximately two years of tamoxifen. In 2020, disease progression prompted treatment with Zoladex and an aromatase inhibitor, later switched to Faslodex due to progression. Subsequent progression with pleural effusion was managed with taxol and trastuzumab, achieving a good response, and the patient was then maintained on trastuzumab plus anastrozole with disease control. Later, brain metastases were treated with whole-brain radiotherapy (two fractions). The regimen was subsequently changed to capecitabine and neratinib for eleven cycles, resulting in regression, and then to neratinib plus tamoxifen. Imaging in January 2026 (CT chest-abdomen-pelvis) demonstrated stable disease extracranially, while brain MRI at the same time showed stable disease. A repeat CT with intravenous contrast in August 2026 indicated stable disease extracranially but possible intracranial progression, with no new lesions noted. Imaging studies reveal multiple supra- and infratentorial intra-axial lesions consistent with cerebral and cerebellar metastases, accompanied by mass effect, as demonstrated on the August 2026 brain MRI. The extracranial disease remains stable on CT imaging. Biomarker assessment confirms HER2 positivity; estrogen and progesterone receptor status, as well as Ki-67, are not documented in the form. The multidisciplinary team recommends continuation of capecitabine and intravenous trastuzumab, with the addition of trastuzumab-deruxtecan (T-DXd) provided through a non-governmental organization.		
تحريراً في ,		
	الوقت :	التاريخ : 23-09-2026
		الطبيب المعالج :
يعتمد ,		



National Women's Health Initiative
Pathology Lab

مركز الباثولوجي النسائي الوطني لصحة المرأة



Pathology Report

Name: **فاطمة إبراهيم محمد عمار** Age: **43 years.**
Clinical data: **Breast cancer (as per the involved report).**
Specimen type: **Slide revision.**
Date (Received): **16 /6/2021.** Date (reported): **13/7/2021**
Lab. Number: **SR-250 / 21** Hospital Number:

Gross Examination:

Ten slides labeled 971/20

- Procedure: Slide revision
- Tumor extent: N/A
 - Number:
 - Size:
 - Skin involvement:
- Distance from margins: N/A
- Lymph nodes number and size: N/A
- Additional pathologic findings: N/A
- Immunohistochemistry technique: N/A

Microscopic examination:

- Invasive duct carcinoma.
- Grade (differentiation + nuclear pleomorphism + mitosis): **G II**
- Angiolymphatic invasion: **N/A**
- Ductal carcinoma in situ: **No**

• Tumor extent: N/A

◦ Number:

◦ Size:

◦ Skin involvement:

◦ Lymph node involvement: N/A

◦ Resection Margins: N/A

◦ Free /DCIS/ invasive carcinoma

• Additional pathologic findings: N/A

Diagnosis:

Breast cancer, Slide revision:

• Invasive duct carcinoma grade II.

• ER: Strong Intensity 90% nuclear reaction (Allred score 8/8)

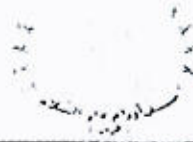
• PR: Strong Intensity 50% nuclear reaction (Allred score 7/8)

• Her2: Strong Intensity >10% membrane reaction (score 3+)

◦ Ki67: 10 % nuclear reaction

Prof. Dr. Manal El Mahdy..

Dr. Momo Mohamed



Patient Name: Hanaa Ibrahim Mohamed Amer	Date : 21/05/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 53 Year(s)
Medical ID: 2798/2021	Pat. ID: 14200

MULTI-SLICE POST CONTRAST CT SCAN OF THE NECK: -

Findings:

- Normal appearance of the nasopharynx, oropharyngeal and laryngeal air spaces.
- No glottic, supra or infra glottic mass lesions.
- Intact cartilagenous skeleton of the larynx.
- Normal appearance of submandibular, parotid salivary glands.
- No neck soft tissue mass lesions.
- Normal size, enhancement & symmetrical appearance of thyroid glands.
- Normal appearance of the neck muscles & vessels.
- No sizable pathologically enlarged cervical lymph nodes.
- Right frontal subcortical intra-axial enhancing SOL with surrounding vasogenic edema measuring 2 cm

OPINION:

- Right frontal SOL for dedicated contrast enhanced MRI.

POST-CONTRAST MULTI-SLICE CT SCAN OF THE BRAIN, CHEST, ABDOMEN & PELVIS: -

***** A reported case of metastatic breast cancer,***

***** Compared to previous non-contrast study dated 23/01/2025***

I. Chest:

• Stationary course regarding:

- Still seen prominent subcentimeteric left internal mammary LN is seen.



Patient Name: Hanaa Ibrahim Mohamed Amer	Date : 21/05/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 53 Year(s)
Medical ID: 2798/2021	Pat. ID: 14200

- Right lower lobe residual few atelectatic/fibrotic bands with focal bronchiectasis.
- Bilateral pulmonary tiny calcific pulmonary nodules (likely sequel of old granulomatous).
- The normal mediastinal fat planes are still seen replaced by soft-tissue sheets.
- No evidence of pleural effusion.
- No gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II. Abdomen and pelvis:

- The liver is enlarged with homogenous CT texture. No focal lesions or biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenal glands.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Stable left adnexal cyst is seen measuring 3.6 x 2.5 cm with mural calcifications.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic lymphadenopathy or collections.
- **Unchanged** several wide-spread sclerotic osseous lesions are seen involving the pelvic bones, few dorso-lumbar vertebrae, few bilateral ribs and left scapula.
- No vertebral structural collapse.

OPINION:

- *Over-all rather stationary course of this study since last examination.*

Dr. Mohamed Mohsen, M.D.



Patient Name: Hanan Ebrahim

Date: 1/1/2021

Tc-99m MDP bone scan.

Desired information : skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP:

Report:

- Active osseous lesions are scattered bilateral at ribs posteriorly, right iliac bone, ?? pelvic bones
- Degenerative changes are noticed in dorso-lumbar vertebrae.
- Arthritic changes over both knee joints.
- Otherwise, the rest of the surveyed skeleton is free.
- Both kidneys are normally visualized.

Opinion: -

- *Suspicious osseous lesions*
- *that warrant radiological correlation as mostly metastases*

Thanks for reference

Huda fathy .M D

Nuclear medicine Unit



Patient Name: Hanaa Ibrahim Mohamed Amer	2025/1/16
Exam : MRI BRAIN WITH CONTRAST	Age : 52 Year(s)
Medical ID: 2798/2021	14200

MRI EXAMINATION OF THE BRAIN REVEALED:

Technique of examination:

- Axial T1, T2 & FLAIR WIs
- Sagittal T1 & Coronal T2 WIs
- Post contrast: axial and sagittal T1 and DWI

**** A reported case of metastatic breast cancer,**

**** Compared to the previous study dated 21/2/2024 (non-contrast),**

MRI Findings:

- Still noted multiple variable sized ill-defined intra-axial lesions seen scattered at both cerebral and cerebellar lobes. They show variable T1 & FLAIR signal, and high T2 signals, with *minimal progression* of the surrounding mild perifocal edema and still causing minimal mass effect in the form of effacement of the overlying cortical sulci.
Some lesions show post-contrast enhancement and other don't. The largest enhancing lesion is seen involving the frontal lobe still measuring about 15x17x14 mm in maximum AP, transverse and craniocaudal dimensions (previous study wasn't contrast-enhanced to ensure precise measurement). While the largest non-enhancing lesion seen at the right cerebellar hemisphere is still measuring 13x18x13mm.
- Still noted bilateral sheets & foci of altered signal intensity of the deep periventricular white matter are noted eliciting patchy high T2 and FLAIR signals.
- Prominent cortical sulci, Sylvian fissures and extra axial CSF spaces.
- Mildly dilated cerebral ventricles with no midline shift or deformity
- No midline shift.

OPINION:

- *Still noted multiple variable-sized intra-axial cerebral & cerebellar metastatic deposits, with minimal progression of surrounding mild perifocal edema as above described.*
- *Small artery disease.*

Dr. Yagoub Saleh

Dr. Sherif Sameh, M.D.